

CLINICAL MULTIPLE CHOICE EVALUATION

SEMINAR 4 OF 20: DEMENTIA AND SAFEGUARDING



Candidate Instructions

- This exam paper contains **25 Single Best Answer (SBA)** questions derived from the seminar course slides.
- All questions are grounded directly in the provided material covering the Care Act (2014), NMC directives, and clinical case studies.
- Select your single best option (**A, B, C, or D**) for each question.
- The official answer key matrix is provided at the absolute end of this document.

QUESTION 1

According to the introductory seminar slides, which specific Nursing and Midwifery Council (NMC) guidance must be combined with "The Code" when addressing clinical issues in practice?

- A) NMC Guidance on Professional Boundaries (2019)
- B) NMC Guidance on Raising Concerns (2025)
- C) NMC Guidelines for Revalidation Cycles (2022)
- D) NMC Delegation Standards for Healthcare Assistants (2020)

QUESTION 2

The course slides outline that the NMC guidance on raising concerns should be utilized in combination with specific local and corporate parameters. Which of the following is explicitly listed?

- A) Whistleblowing policies issued by your employer
- B) Financial audit templates compiled by ward administrators
- C) The individual nurse's employment contract log
- D) National pharmaceutical manufacturing directories

QUESTION 3

How is 'Safeguarding' explicitly defined in the core text of your course slides?

- A) To optimize the economic output of regional trusts while managing staff resource allocations
- B) To protect an individual's health, wellbeing and human rights; enabling them to live free from harm, abuse and neglect
- C) To provide legal defense indemnity documentation for senior clinical care management tiers
- D) To coordinate outpatient appointment timelines to meet national target frameworks

QUESTION 4

The lecture slides define 'Abuse' as a violation of an individual's human and civil rights. What three explicit alternative drivers or causes behind abuse are named?

- A) Malice, strategic intent, structural bias
- B) Deliberate intent, negligence or ignorance
- C) Cognitive impairment, resource scarcity, physical strain
- D) Environmental failure, systematic pressure, poor oversight

QUESTION 5

According to the legal frameworks listed on Slide 6, which piece of legislation sets out the clear legal framework for how we should protect adults at risk of abuse or neglect?

- A) The Mental Capacity Act 2005
- B) The Care Act (2014)
- C) The Human Rights Act 1998
- D) The Equality Act 2010

QUESTION 6

Under the Care Act (2014) statutory criteria, safeguarding adults applies explicitly to any adult who meets which specific set of factors?

- A) Is over the age of 65 and resides within a residential care setting independently
- B) Has care and support needs, is experiencing or at risk of abuse/neglect, and is unable to protect themselves because of those needs
- C) Lacks formal cognitive processing capacity as determined by two separate medical consultants
- D) Requires acute mechanical ventilation and is under full public financial funding streams

QUESTION 7

How many distinct core principles are addressed within the framework of The Care Act (2014) in your seminar slides?

- A) Four principles
- B) Six principles
- C) Eight principles
- D) Ten principles

QUESTION 8

Which of the Care Act principles is defined exactly as: "patients should be supported and encouraged to make their own decisions"?

- A) Protection
- B) Empowerment
- C) Proportionality
- D) Partnership

QUESTION 9

Under The Care Act (2014) framework, how is the principle of 'Prevention' explicitly defined?

- A) The response should be the least intrusive and most appropriate to the risk presented
- B) It is better to take action before harm occurs rather than waiting until it does
- C) There should be dedicated support pathways for those in the greatest social need
- D) All clinical safeguarding practice should be fully transparent and audited

QUESTION 10

Which principle dictates that a safeguarding response "should be the least intrusive and most appropriate to the risk"?

- A) Accountability
- B) Proportionality
- C) Partnership
- D) Empowerment

QUESTION 11

How many distinct types of abuse are explicitly tabulated in the course text on Slide 8?

- A) 7 types
- B) 10 types
- C) 12 types
- D) 15 types

QUESTION 12

Which category of abuse from your curriculum list directly encompasses "acts of omission"?

- A) Organisational or institutional abuse
- B) Neglect or acts of omission
- C) Discriminatory abuse
- D) Financial or material abuse

QUESTION 13

The slides describe that incidents of abuse typically come to light through four specific mechanisms. Which mechanism is explicitly listed?

- A) Random local pharmacy stock audits
- B) Disclosure, witnessing an incident, observing signs/symptoms, or online platforms
- C) Reviewing patient length-of-stay parameters across the trust dashboard
- D) Standard cross-checking of medical fee codes

QUESTION 14

According to Slide 11, under what explicit circumstance must a clinician immediately call 999?

- A) If a patient's relative requests access to paper chart summaries
- B) If the person is in immediate danger or needs urgent medical attention
- C) If a historical financial discrepancy is disclosed by an administrative colleague
- D) If a student nurse requests a change to their clinical placement log

QUESTION 15

If a clinical situation is not urgent but you firmly believe a crime has taken place, what specific action is directed on Slide 11?

- A) E-mail the Care Quality Commission via their standard notification log
- B) Phone the police on 101
- C) Wait until the close of the full weekly shift block to file a hardcopy file
- D) Conduct an independent investigation alongside adjacent ward staff

QUESTION 16

In Case Scenario 1, a 72-year-old man is admitted for dementia assessment. His carer reports a history of falls, but your examination reveals what specific anomaly?

- A) An acute fracture that requires immediate surgical fixation pinning
- B) Bruises which are not consistent with falling
- C) An unexpected rapid improvement in short term spatial processing scores
- D) Significant dehydration accompanied by severe stage 4 pressure ulcers

QUESTION 17

In Case Scenario 2, Mrs. Jones is given a sedative injection by the nurse in charge every single time she shouts out "to calm her down". This pattern of practice raises major flags for what type of abuse?

- A) Financial exploitation
- B) Organisational or institutional abuse (involving potential chemical restraint)
- C) Self-neglect progression
- D) Discriminatory profiling exclusively

QUESTION 18

During the events of Case Scenario 2, what explicit instruction does the nurse in charge give you regarding Mrs. Jones's lunchtime care?

- A) To document her food intake parameters within the electronic system immediately
- B) To leave the food on the side and not wake the patient up as she would start shouting again
- C) To transfer her care directly to an auxiliary volunteer or family member
- D) To prepare an additional verbal SBAR briefing before approaching the bed locker

QUESTION 19

In Case Scenario 3, a day-shift colleague explains that they left medications on a table for dementia patients to take independently, stating: "they don't know anyway what they take or when". Why is this an explicit safeguarding and abuse issue?

- A) Because medications can only be distributed by pharmacy consultants legally
- B) It represents severe neglect or acts of omission, posing immediate clinical risk to vulnerable adults
- C) Because the patient's family members did not co-sign the tracking receipt sheet
- D) It alters the ward budget parameters for scheduled controlled substance boxes

QUESTION 20

Slide 17 itemizes four key legal and ethical considerations in dementia and safeguarding. Which consideration is listed second?

- A) Informed consent and treatment decisions
- B) Lasting Power of Attorney
- C) Privacy and confidentiality
- D) End of Life Care and Advance Care Planning

QUESTION 21

Which independent support helpline number is explicitly provided on your slide resources for contacting the Alzheimer's Society Dementia Connect service?

- A) 0800 678 1602
- B) 0333 150 3456
- C) 0800 319 6789
- D) 0800 888 6678

QUESTION 22

What helpline contact is provided across Slide 18 to directly reach the Admiral Nurse Dementia Helpline at Dementia UK?

- A) 0800 888 6678
- B) 0800 808 7777
- C) 0800 652 1102
- D) 0800 678 1602

QUESTION 23

According to the seminar reference list, which specific section of the Mental Capacity Act 2005 is highlighted as vital reading for this module?

- A) Section 5
- B) Section 21
- C) Section 44
- D) Section 60

QUESTION 24

The seminar slides state that when a nurse is safeguarding health and wellbeing, they must ensure patients are not exposed to abuse or neglect. This is tied structurally to which overarching framework?

- A) The local trust payroll structure
- B) The NMC Standards of Proficiency and The Code
- C) The international clinical triaging indexing system
- D) The regional acute funding matrix documentation

QUESTION 25

Which resource mapping is explicitly listed on your references slide for charting safeguarding processes systematically?

- A) The Greater Manchester Medication Formulary
- B) The SCIE process map
- C) The Waterlow Risk Assessment Card
- D) The Datix Adverse Event Reporting Grid

 OFFICIAL EXAM ANSWER KEY MATRIX

Cross-reference candidate answer selections against this strict verification matrix block.

Q1	Q2	Q3	Q4	Q5	Q6	Q7	Q8	Q9	Q10
B	A	B	B	B	B	B	B	B	B

Q11	Q12	Q13	Q14	Q15	Q16	Q17	Q18	Q19	Q20
B	B	B	B	B	B	B	B	B	B

Q21	Q22	Q23	Q24	Q25
B	A	C	B	B